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CRITICAL CARE

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STRESS ULCER PROPHYLAXIS IN ICU

REVIEW BY (MAY 2024)

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Key Points

- Critically Ill patients are at risk from stress ulceration and subsequent clinically significant bleeding.
- Certain risk factors will increase the risk of bleeding
- ICU patients should be started on stress ulcer prophylaxis (SUP) if indicated
- Choice of agent is dependent on patient condition

GIVE STRESS ULCER PROPHYLAXIS (SUP)

IF:

ONE or more of the risk factors below:

- **Bleeding diathesis (eg Platelet count <50,00 per m^3, an international normalised ratio (INR) of >1.5 or a partial thromboplastin time (PTT) >2 times the control**
- **Mechanical ventilation >48hours/respiratory failure**
- **History of GI ulceration or GI bleeding within the past year**
- **Traumatic brain injury, traumatic spinal cord injury or burns injury**

OR

TWO or more of the risk factors below:

- **Sepsis**
- **ICU stay more than a week**
- **Occult GI bleeding for 6 or more days**
- **Glucocorticoid therapy (more than 250mg Hydrocortisone or the equivalent)**
- **On non-steroidal anti-inflammatory or anti-platelet agents**

For all others, omit SUP but review need daily

TREATMENT REGIME

CONDITION	SUGGESTED TREATMENT
Full NG/Oral feeds	PO PPI : Lanzoprazole 30mg OD
Not on NG feeds or not absorbing	IV PPI : Omeprazole 40mg OD
Not tolerating PPI	PO/IV Ranitidine (based on feeds above)
Not tolerating PPI/H2 blocker	Sucralfate